

BioNixus

intelligence for Business Growth

SAMPLE EDITION · AUGUST 2026

GCC Healthcare Market Report 2026

Country-by-country intelligence for Saudi Arabia, the UAE, Qatar, Kuwait, Oman and Bahrain — pharmaceutical, medical devices, regulation, procurement and hospital access.

6

GCC MEMBER STATES

~\$27B

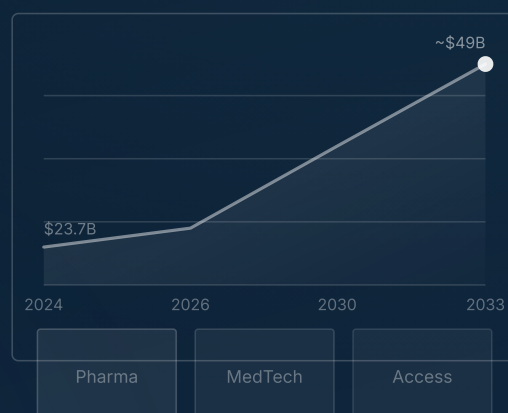
PHARMA MARKET 2026

\$9.0B

MEDTECH & IVD 2026

~62M

COMBINED POPULATION



COVERAGE · 6 GCC STATES

FOCUS · PHARMA · MEDTECH · ACCESS

USE · LAUNCH & TENDER PLANNING

01

HOW TO READ THIS SAMPLE

Contents

This PDF is a gated sample of BioNixus GCC healthcare intelligence, aligned to figures published on bionixus.com country and regional reports and validated against cited public sources (August 2026). Country chapters are not snapshots — each market has its own size, regulator, tender system and hospital map. The full databook and custom primary research are scoped separately.

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About BioNexus

WHAT THIS SAMPLE IS — AND IS NOT

It is a board-ready briefing drawn from BioNexus published market intelligence: sizing bands, regulators, access steps, epidemiology and named hospital systems. It is **not** a syndicated SKU database, not a tender award dump, and not medical, legal or investment advice. Ranges reflect triangulation across BioNexus estimates and cited secondary sources — the published BioNexus GCC medical devices rollup for devices, and the IDF Diabetes Atlas (11th edition, 2024 estimates) for diabetes prevalence.

Full country reports: [bionixus.com/saudi-arabia-healthcare-market-report](#) · [/uae-healthcare-market-report](#) · [/qatar-healthcare-market-report](#) · [/kuwait-healthcare-market-report](#) · [/oman-healthcare-market-report](#) · [/bahrain-healthcare-market-report](#) · [/gcc-pharma-market-report-2026](#) · [/gcc-medical-devices-market-report](#) · [/gcc-market-access-guide](#)

02

REGIONAL SNAPSHOT

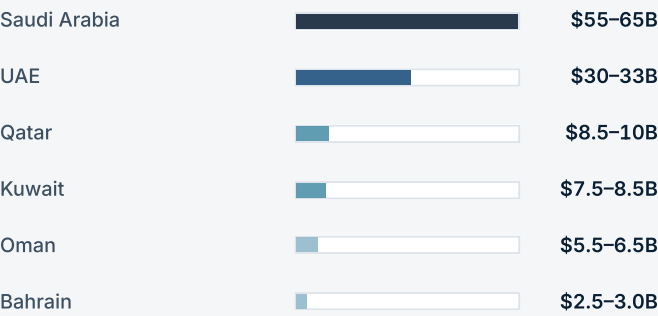
GCC at a glance

The six GCC states — Saudi Arabia, the UAE, Qatar, Kuwait, Oman and Bahrain — remain sovereign markets. Gulf Health Council dialogue does not replace national marketing authorisations. Hydrocarbon fiscal capacity, Vision-era hospital build-out, and among the world's highest diabetes prevalence rates concentrate specialty spend in oncology, metabolic disease, cardiovascular and biologics.

~62M Population (6 states) Saudi Arabia 37.2M · UAE 10.3M · Kuwait 4.8M · Oman ~5.0M · Qatar 2.84M · Bahrain 1.63M	~\$27B Pharma market 2026 From USD 23.7B in 2024 at 7.6% CAGR — BioNexus GCC pharma market analysis, 2026	\$9.0B MedTech / IVD 2026 Secondary rollup cited in the BioNexus GCC medical devices report (August 2026)	6.7% Devices CAGR to 2035 Reaching USD 16.2B by 2035 — capital equipment, consumables and IVD scope
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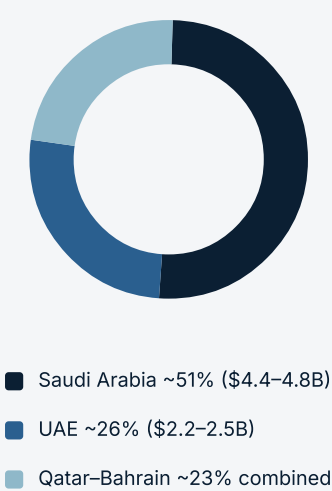
Healthcare market size by country, 2026

USD billions — published BioNexus country healthcare market reports (bands shown at midpoint)



Where device spend sits

Share of the USD 9.0B GCC medical devices market, 2026



Device shares: BioNexus GCC medical devices report rollup (\$9.0B → \$16.2B by 2035). Pharma rollup: BioNexus GCC pharma market analysis 2026 — Saudi Arabia ~46% of GCC pharma spend; KSA + UAE together roughly two-thirds of the Gulf total.

Healthcare and pharma bands by country — 2026, published BioNexus country reports

COUNTRY	HEALTHCARE SPEND	PHARMA HEADLINE	SYSTEM
Saudi Arabia	USD 55–65B	USD 8.5–9.5B	SFDA · NUPCO · Vision 2030
United Arab Emirates	USD 30–33B	USD 3.8–4.5B	MOHAP · DHA · DOH
Qatar	USD 8.5–10B	USD 900M–1.05B	MOPH · HMC / HHC
Kuwait	USD 7.5–8.5B	USD 950M–1.1B	MOH / DGPA · CMS
Oman	USD 5.5–6.5B	USD 520–580M	MOH · Central Pharmacy
Bahrain	USD 2.5–3.0B	USD 420–480M	NHRA · MOH tenders

Country healthcare and pharma bands are taken from each BioNexus country healthcare market report (May 2026 update). UAE pharma band is the BioNexus market-intelligence KPI (USD 3.8–4.5B). The GCC pharma rollup (~USD 27B, 2026) uses a broader full-channel scope — including retail, OTC and consumer-health volume — and is *not* a simple sum of the narrower country prescription bands above. See Methodology for reconciliation rules.

GCC pharmaceutical market

BioNexus values the six-country GCC pharmaceutical market at roughly USD 23.7 billion in 2024, growing at about 7.6% CAGR — near USD 27 billion in 2026, on track for approximately USD 37 billion by 2030 and close to USD 49 billion by 2033. Saudi Arabia accounts for around 46% of regional spend; with the UAE second, the two markets carry roughly two-thirds of the Gulf total.

\$23.7B**GCC pharma 2024**

BioNexus market analysis,
2026

~\$27B**GCC pharma 2026**

Carried forward at 7.6%
CAGR

~\$37B**Projected 2030**

~USD 49B by 2033

~46%**Saudi share of spend**

KSA + UAE ≈ two-thirds of
the Gulf total

Hospital tenders — **NUPCO** in Saudi Arabia, **HMC / Health Holding Company** in Qatar, **Central Medical Stores** in Kuwait, and **MOH central stores** in Oman and Bahrain — still dominate public volume; insurance and retail matter most in the UAE and in Saudi Vision 2030 privatisation corridors. Qatar and Kuwait punch well above their population weight on a per-capita basis, reflecting high public health budgets and near-universal coverage.

STRUCTURAL DRIVERS

What is moving the market

- Vision 2030's SAR 500 billion healthcare investment programme in Saudi Arabia; UAE Vision 2031 medical tourism and mandatory insurance
- GLP-1 and oncology specialty intensity — GLP-1 prescriptions growing at 40%+ annually in Saudi Arabia
- Biosimilar tendering (adalimumab and others) resetting net price after 14 SFDA biosimilar approvals in 2023–2025
- Localisation and JV manufacturing KPIs — Saudi Arabia targets 40% local pharmaceutical manufacturing by 2030

ADJACENT GCC PHARMA CUTS, 2026

Published segment sizing

- **Biologics:** USD 4.8–5.4B (Saudi ~55%, UAE ~20%); Saudi biologics tracking from ~\$6.8B (2022) toward ~\$14B by 2030
- **Generic injectables:** USD 1.6–1.9B, shaped by NUPCO and MOHAP hospital volume
- **Immunology & biologics:** ~USD 5.9B (2026) → ~USD 9.7B by 2030 at ~12.6% CAGR
- **Generics:** heading toward ~USD 15.9B by 2033; biosimilars growing at 15–20% CAGR
- **Precision medicine:** fastest Saudi subsegment — Saudi Genome Program corridors

Country splits and import dependency — GCC pharma rollup scope

MARKET	2024 SIZE (EST.)	CAGR 2024–26	PER-CAPITA POSITION	IMPORT DEPENDENCY
Saudi Arabia	~\$11B	~7.5%	Moderate (largest absolute)	~70–80%
UAE	~\$4.5B	~8.2%	High	~80%
Kuwait	~\$1.9B	~4.8%	Very high per-capita	~80%
Qatar	~\$1.6B	~6.1%	Highest per-capita	~97%
Oman	~\$1.4B	~5.0%	Moderate	~95%
Bahrain	~\$0.9B	~4.2%	Cost-sensitive	~91%

Source: BioNixus GCC Pharma Market Report 2026 (full-channel scope). High import dependency across every Gulf market is exactly what the localisation agenda targets: as Saudi Arabia and the UAE scale domestic manufacturing, tender preference and net pricing shift toward regionally produced volume.

Access is sequential — not a single GCC licence

- 01

National marketing authorisation — SFDA, MOHAP, MOPH, NHRA or MOH

Typical technical review of 6–30 months by country and product class. The GCC Centralised Registration Procedure and Gulf Health Council harmonisation help sequencing but do not replace the national dossier, pricing file or tender entry.
- 02

Price approval — SFDA Pricing, MOHAP Pricing Committee, MOPH, MOH pricing committees

External reference pricing dominates: Saudi Arabia benchmarks against reference-country baskets via the NEPA price database; UAE references include KSA, Jordan, France and the UK. Typically 2–6 months post-technical clearance. Reference baskets differ by country.
- 03

Central procurement — NUPCO (KSA), HMC/HHC (Qatar), CMS (Kuwait), MOH tenders (Bahrain, Oman)

INN-based single-winner lots are standard in Saudi Arabia, Qatar and Kuwait — the tender outcome for a molecule can define effectively all public-sector volume until the next cycle.
- 04

Private payer / hospital P&T — CCHI, Daman, AXA Gulf, Allianz Care, Bupa Arabia, private hospital committees

A parallel track that is essential for UAE dual-emirate launches and increasingly material in Saudi privatisation corridors, Oman (Dhamani) and Bahrain (Takaful and expatriate mandates).

BIONIXUS RECOMMENDATION

File **SFDA and MOHAP in parallel** when the commercial plan requires simultaneous KSA and UAE access. Sequencing only through one "reference" GCC agency still leaves pricing and tender clocks untouched in the second market.

Sources: BioNixus GCC Pharma Market Report 2026 (/gcc-pharma-market-report-2026); GCC biologics & generic injectables intelligence (/gcc-pharmaceutical-market-research); GCC market access guide (/gcc-market-access-guide).

GCC medical devices market

The GCC medical devices market is USD 9.0 billion in 2026, projected to USD 16.2 billion by 2035 at approximately 6.7% CAGR — the secondary rollup cited in the BioNexus GCC medical devices report (August 2026 update). Scope is capital equipment, consumables and IVD. Competing vendor totals diverge when digital health, dental-only or refurbished channels are folded in.

\$9.0B

Market size 2026

Capital equipment + consumables + IVD

\$16.2B

Forecast 2035

Fastest-growing major devices region on a percentage basis

6.7%

CAGR 2026–2035

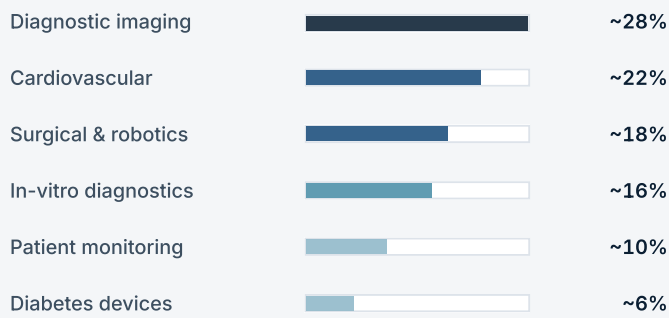
Saudi Arabia + UAE ≈ 77% of regional device spend

COUNTRY	2026 DEVICES	2035 FORECAST	CAGR	DEVICE REGULATOR
Saudi Arabia	USD 4.4–4.8B (~51%)	USD 8.0–8.8B	~6.9%	SFDA Medical Devices Sector (MDIR Class A–D)
United Arab Emirates	USD 2.2–2.5B (~26%)	USD 4.3–4.9B	~7.6%	MOHAP + DHA / DOH emirate pathways
Kuwait	USD 750–850M	USD 1.1–1.25B	~4.3%	MOH Kuwait · CMS procurement (~85% of volume)
Qatar	USD 680–760M	USD 1.2–1.4B	~6.5%	MOPH · HMC procurement evaluation
Oman	USD 280–320M	USD 390–440M	~3.9%	MOH Oman
Bahrain	USD 220–280M	USD 300–360M	~3.5%	NHRA (mutual-recognises SFDA / MOHAP clearances)

Source: bionexus.com/gcc-medical-devices-market-report (updated August 2026). Country shares roll up to the \$9.0B GCC total. Some country healthcare pages still carry older device KPI bands; this sample uses the GCC devices report as the regional source of truth.

Segment mix — share of market value

BioNixus GCC medical devices report, 2026



Fastest-growing device categories

Ranked by BioNixus procurement and clinical-user research

- 1 · Minimally invasive / robotic surgery — accelerating at KFSH&RC and Cleveland Clinic Abu Dhabi
- 2 · Point-of-care diagnostics — HbA1c, cardiac biomarkers, infectious disease rapid tests
- 3 · Continuous glucose monitoring — reimbursement expanding in KSA and UAE
- 4 · Connected / remote patient monitoring
- 5 · AI-powered imaging driving diagnostic upgrade cycles

DEVICE ACCESS, COMPRESSED

Registration to revenue

Classify risk and appoint a GCC authorised representative → technical file / ISO 13485 and Arabic IFU as required → country marketing authorisation (6–18 months by class; Class A notification faster) → public procurement / CMS / hospital biomedical evaluation (3–12 months) → private hospital and insurer adoption (2–9 months). NHRA Bahrain may mutual-recognise SFDA and MOHAP clearances; parallel SFDA + MOHAP filings accelerate combined KSA + UAE launches.

OEM LANDSCAPE

Who supplies the region

Imaging: Siemens Healthineers, GE Healthcare, Philips, Mindray (growing mid-tier share). Cardiovascular: Medtronic, Abbott, Boston Scientific, Edwards Lifesciences. Surgical and orthopaedics: Intuitive Surgical, Stryker, Zimmer Biomet, Johnson & Johnson MedTech. Local distribution runs through registered Saudi and UAE agents including Al-Faisaliah, AMSCO and Cigalah.

Therapy & disease landscape

The Gulf's therapy mix is set by an unusually heavy metabolic and cardiovascular burden, concentrated tertiary oncology capacity, and payer systems that fund premium biologics. Epidemiology below is validated against the IDF Diabetes Atlas 11th edition (2024 estimates) and national statistics.

Adult diabetes prevalence — age-standardised, 2024

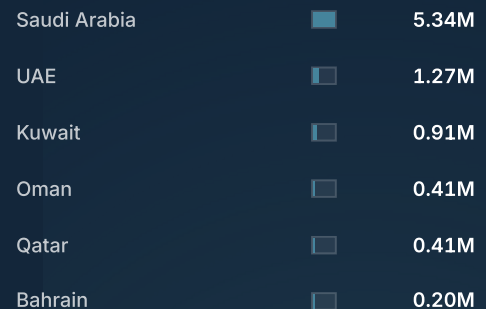
IDF Diabetes Atlas, 11th edition (adults 20–79)



Kuwait ranks 3rd globally; Qatar 5th; Saudi Arabia 7th; Bahrain 10th. Undiagnosed share runs from 37% (Kuwait) to 64% (UAE) — a screening and diagnostics demand signal in its own right.

People living with diabetes, 2024

Adults 20–79 — IDF Diabetes Atlas 11th edition



This is not a "lifestyle footnote"; it is the GLP-1, SGLT-2, insulin and CGM demand engine for the whole region.

ONCOLOGY

Fastest-growing GCC therapy area by spend

~35,000+ new GCC diagnoses per year combined (national registries); Saudi Cancer Registry alone records ~25,000 new cases annually, with colorectal (males) and breast (females) most prevalent. Capacity anchors: KFSH&RC, HMC's NCCCR, Cleveland Clinic Abu Dhabi and Sidra Medicine CAR-T / genomics programmes. Saudi oncology spend is highly concentrated — the top 15 hospital accounts carry a disproportionate share of Kingdom oncology consumption.

METABOLIC & CARDIOVASCULAR

The volume backbone

Cardiovascular disease is the leading cause of death in every GCC state — roughly 28–38% of mortality (Kuwait ~38%, Saudi ~35%, Bahrain 32%, Oman 28%). Obesity compounds the metabolic burden: 47% of Kuwaiti adults, 42% in Qatar (highest GCC), 37% in the UAE. NOACs, SGLT-2 cardiac indications, GLP-1 intensification and lipid management dominate prescription volume.

IMMUNOLOGY, BIOLOGICS & RARE DISEASE

Where the premium sits

GCC immunology and biologics spend is ~USD 5.9B in 2026, heading toward ~USD 9.7B by 2030 (~12.6% CAGR). Adalimumab biosimilar NUPCO tenders are reshaping net price. Elevated consanguinity in Kuwait, Bahrain and Saudi Arabia creates concentrated rare-disease and genomics demand — the commercial logic behind the Saudi Genome Program corridors.

Ramadan dosing shifts, Hajj seasonal surges in respiratory and pulmonology demand, migrant uninsured segments (especially UAE construction corridors), and TB-screening delays before first biologic are real utilisation variables. BioNixus measures them in primary HCP and hospital analogue work.

Sources: IDF Diabetes Atlas 11th edition (2024 estimates); MOH national statistics 2023–2024; national cancer registries 2022–2023; Qatar STEPS Survey 2022; UAE NCD Survey 2022; WHO Kuwait Country Profile 2022; BioNixus GCC market intelligence therapy segments.

06

MARKET ACCESS

Regulatory & procurement map

Six sovereign regulators, six pricing files, six tender systems. The table below maps each market's pharma and device authorities, the central procurement channel that controls public volume, and the realistic launch-to-access clock from first submission to funded prescription.

COUNTRY	PHARMA REGULATOR	DEVICE REGULATOR	CENTRAL PROCUREMENT	LAUNCH → ACCESS
Saudi Arabia	SFDA	SFDA MDIR	NUPCO (+ NGHА parallel captive)	36–54 months
UAE	MOHAP	MOHAP / DHA / DOH	Emirate formularies + insurers	18–30 months
Qatar	MOPH	MOPH	HMC / Health Holding Co.	18–36 months
Kuwait	MOH / DGPA	MOH	Central Medical Stores	24–45 months
Oman	MOH Drug Control	MOH	MOH Central Pharmacy / CSSD	24–45 months
Bahrain	NHRA	NHRA	MOH Central Pharmacy	18–30 months

Launch-to-access clocks are the published BioNixus country market access timelines (regulatory approval + payer listing + formulary/tender access). Registration-only timelines are shorter — e.g. SFDA 8–18 months, NHRA 6–12 months standalone or 3–9 months via mutual recognition.

Saudi Arabia: SFDA introduced a mandatory Economic Evaluation System (EES) from 1 July 2025, raising the bar for pharmacoeconomic and budget-impact evidence at registration. Build the EES file into the registration plan, not as a post-MA surprise.

UAE: innovators still version dossiers for DHA (Dubai) and DOH (Abu Dhabi) in parallel with federal MOHAP marketing authorisation — three regulatory environments, one launch.

Qatar: concentrates high-cost oncology behind HMC global budgets and the MOPH National Drug Committee's health-economic review; Sidra Medicine runs a mandatory parallel formulary for paediatric and genomic indications.

Kuwait: CMS single-winner tenders make price the dominant public-channel criterion; KOC and Kuwait Airways run smaller parallel employee formularies.

Bahrain: NHRA's lean review and GCC mutual-recognition pathway is the fastest small-market on-ramp — but pricing and MOH tenders remain fully national.

INSURANCE ARCHITECTURE, SIMPLIFIED

Who pays where

- **UAE:** mandatory insurance (Abu Dhabi Thiqa since 2007; Dubai since 2014; federal expansion ongoing). Private share structurally highest in the GCC at ~65% of healthcare expenditure.
- **Saudi:** mandatory for non-nationals since 2006, expanding to nationals under Vision 2030 / CCHI.
- **Qatar & Kuwait:** public-dominant (~80% / ~78% of spend) with private carve-outs — Seha scheme direction in Qatar, Al-Afya expatriate coverage in Kuwait.
- **Oman:** Dhamani mandatory private-sector insurance building a second access channel under Vision 2040.
- **Bahrain:** government ~65% of spend; expatriate employer mandates and Takaful expanding private reimbursement.

PRICING MECHANISMS

How prices are set

- **Saudi Arabia:** external reference pricing against a reference-country basket; NEPA price database.
- **UAE:** ERP against EU + regional basket (incl. KSA, Jordan, France, UK).
- **Qatar:** reference pricing benchmarked to KSA, UAE and select EU markets.
- **Kuwait:** cost-plus with manufacturer price ceiling.
- **Bahrain:** ERP under the GCC harmonised pricing initiative; first GCC state with a formal biosimilar pathway.
- **Oman:** GCC unified pricing framework (in progress).

07

COUNTRY 1 OF 6

Saudi Arabia

The largest healthcare and pharmaceutical market in the Arab world and the GCC. The healthcare market is estimated at USD 55–65 billion in 2026, on a path toward USD 90–110 billion by 2030 as Vision 2030's SAR 500 billion healthcare programme adds capacity, private participation and insurance-funded consumption.

37.2M

Population 2026

World Bank estimate · GDP per capita USD 26,800 (IMF 2025)

\$55–65B

Healthcare market

→ USD 90–110B by 2030 · government ~62% / private 38% and rising

\$8.5–9.5B

Pharma 2026

Hospital channel ~42% of spend through NUPCO · ~7.5% CAGR

\$4.4–4.8B

Devices 2026

~51% of GCC device spend — GCC devices report band

Vision 2030 targets: 35% private healthcare participation; 40% local pharmaceutical manufacturing by 2030; 30,000+ additional hospital beds. Government spend is about 62% of total health expenditure. Oncology, diabetes, cardiovascular and immunology/biologics are the four largest therapy areas; GLP-1 is the fastest individual class, with prescriptions growing 40%+ annually on the published country report.

Therapy value — BioNexus 2026

AREA	SIZE 2026	CAGR	ANCHOR
Oncology	USD 1.2–1.4B	12%	KFSH&RC; 14+ new cancer centres by 2030
Diabetes & metabolic	USD 950M–1.1B	14%	23.1% age-standardised adult T2DM (IDF 11th ed.); GLP-1 surge
Cardiovascular	USD 1.0–1.2B	10%	NGHA cardiac volumes; NOACs
Immunology & biologics	USD 700–850M	11%	Adalimumab biosimilar net-price reset
Respiratory	USD 500–650M	9%	Hajj pulmonology; asthma biologics

The SFDA → NUPCO clock

- 01

SFDA pre-submission meeting — 4–8 weeks

Clarifies dossier requirements and the reference-country pricing set.
- 02

eCTD dossier, Modules 1–5, Arabic + English → technical review

18–30 months for an innovative NME; 12–18 months for generics and biosimilars. GMP certificate required; reference-country data accepted.
- 03

SFDA pricing — 3–6 months post-technical clearance

Benchmarked against the SFDA reference-country basket via the NEPA price database. Marketing authorisation issued, valid 5 years.
- 04

NUPCO formulary uplift dossier — 6–12 months post-MA

Health-economic dossier required for innovative products; Arabic submission. MOH National Drug Formulary Committee adds 3–6 months, including biosimilar interchangeability decisions.
- 05

NUPCO annual INN tender — 3–9 months

Single national winner per product category. Full launch-to-access: **36–54 months**.

ACCESS REALITY

NUPCO governs MOH lots across ~240+ hospital accounts; **NGHA runs a partially parallel captive**. Private syndicates compress net price but keep premium biologic carve-outs. The SFDA Economic Evaluation System (mandatory from 1 July 2025) should be in the registration plan, not a post-MA surprise.

SYSTEM CAPACITY

Beds, physicians, hospitals

- Total health expenditure USD 60–65B (~6.5% of GDP); USD 1,640 per capita
- ~70,000 beds (1.9 per 1,000) · ~95,000 physicians (2.6 per 1,000)
- 500+ hospitals — MOH ~287, private ~185, other government ~80
- Epidemiology: T2DM 23.1% age-standardised, 7th highest globally (IDF 11th ed., 2024); CVD ~35% of deaths (MOH 2024); cancer ~25,000 new cases/year, colorectal (M) and breast (F) lead (Saudi Cancer Registry 2023)

HOSPITALS THAT SET THE ANALOGUE

Reference accounts

KFSH&RC Oncology, transplant, CAR-T, genomics	1,200 beds
KAMC / NGH Largest government network	1,500 beds
King Salman Hospital, Riyadh Tertiary general	1,500 beds
KFMC Oncology, paediatrics, neurosciences	1,100 beds
Dr Sulaiman Al Habib (HMG) Largest private group	3,000+ beds
Saudi German · Mouwasat Multi-site private tertiary	multi-site

Pfizer

Roche

Novartis

AstraZeneca

MSD

Sanofi

AbbVie

Eli Lilly

Novo Nordisk

J&J

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08

COUNTRY 2 OF 6

United Arab Emirates

The GCC's second-largest healthcare market and its fastest-growing pharmaceutical market. Estimated at USD 30–33 billion in 2026, on a path toward about USD 60 billion by 2030 (7–8% CAGR). Split: Abu Dhabi ~45%, Dubai ~40%, other emirates ~15%. Private spend is roughly 65% of healthcare expenditure — the GCC's most insurance-driven mix.

10.3M

Population · ~89% expats

GDP per capita ~USD 50,000 (IMF 2025)

\$30–33B

Healthcare 2026

→ ~USD 60B by 2030 · THE band USD 32–36B in market-intelligence KPIs (~USD 3,200 per capita)

\$3.8–4.5B

Pharma 2026

Second in the GCC — BioNixus market-intelligence KPI

\$2.2–2.5B

Devices 2026

~26% of the GCC \$9.0B rollup · Dubai is the regional distribution hub

Dual regulation is the commercial fact: MOHAP issues federal marketing authorisation; DHA (Dubai) and DOH (Abu Dhabi) still govern emirate facility, formulary and pharmacovigilance overlays. Medical tourism exceeded 350,000 patients in

2025 on the published UAE report, concentrating premium demand in oncology, cardiac, orthopaedic and reproductive medicine.

AREA	SIZE 2026	CAGR	ANCHOR
Oncology	USD 450–520M	11.5%	Cleveland Clinic Abu Dhabi, Burjeel Medical City, American Hospital Dubai
Cardiovascular	USD 400–460M	11.7%	Medical tourism cardiac; TAVI / MitraClip volumes
Diabetes & metabolic	USD 380–440M	14%	20.7% age-standardised adult T2DM; 37% adult obesity
Immunology & biologics	USD 320–380M	12%	High-income expat + private payer coverage
Ophthalmology	USD 180–240M	10%	LASIK, cataract, AMD; MENA hub

MOHAP, DHA, DOH — and who pays

Federal MA: eCTD, Arabic labelling, GCC CTD accepted. Technical review 12–18 months (innovative) or 6–12 months (generic); EMA / FDA / Health Canada approvals can abridge. Price committee 2–4 months (references include KSA, Jordan, France, UK). Then separate DHA and DOH facility / formulary tracks (4–8 weeks each post-MOHAP) plus insurer listing (Daman, AXA Gulf, Allianz Care, Bupa Arabia — 2–6 months). Total launch-to-access: **18–30 months** if DHA and DOH run in parallel.

PAYER TEXTURE Formulary listing, not registration, determines access Thiqa covers Abu Dhabi nationals with rich floors; Dubai's Essential Benefits Plan scaffolds lower-income expatriates; DIFC employer captives add a third rule set. DOH value-based pilots can bundle diabetes drug spend with HbA1c outcomes — GLP-1 ceilings are not tender unit-price alone. Formulary inclusion varies significantly by payer across hundreds of private insurers.	CAPACITY & EPIDEMIOLOGY System profile • ~9,500 beds (0.9 per 1,000 — medical tourism augments) · ~25,000 physicians (2.4 per 1,000) · 150+ hospitals (public 50+, private 100+) • T2DM 20.7% age-standardised (IDF 11th ed., 2024) — 64% of cases undiagnosed, the region's highest • Obesity 37% of adults (UAE NCD Survey 2022) • Cancer ~5,000 new cases/year (UAE National Cancer Registry 2023)
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HOSPITALS

Reference accounts

Cleveland Clinic Abu Dhabi **364 beds**

Oncology, cardiology, neurology — JCI

Burjeel Medical City **450 beds**

Oncology, orthopaedics, transplant

SKMC — Abu Dhabi **700 beds**

Main public tertiary / trauma

Dubai Hospital **650 beds**

Main Dubai public tertiary

Tawam Hospital, Al Ain **487 beds**

Oncology reference centre

American Hospital Dubai · Mediclinic City **254 / 280 beds**

Private, JCI-accredited

COMMERCIAL INTELLIGENCE

Three UAE realities

- Multi-pathway registration is the primary bottleneck — a MOHAP registration does not automatically open Dubai or Abu Dhabi government formularies.
- Biosimilar penetration is among the GCC's highest, especially in rheumatology and gastroenterology where DOH has published substitution guidance.
- Medical tourism creates premium demand beyond the resident population, at price points above standard UAE reimbursement.

bionixus.com/uae-healthcare-market-report · /uae-medical-devices-market-report

09

COUNTRY 3 OF 6

Qatar

Estimated at USD 8.5–10 billion in 2026 (USD 14–16 billion by 2030 on the country report). Healthcare is roughly 4% of GDP, but per-capita spend of ~USD 3,000–3,500 is among the world's highest. Government dominates ~80% of spend; Hamad Medical Corporation is the system operator, and Hamad General hosts one of the region's largest hospital pharmacies.

2.84M

Population 2026

GDP per capita USD 85,000–90,000 — highest in the GCC

\$8.5–10B

Healthcare 2026

→ USD 14–16B by 2030 · government ~80% of spend

\$0.90–1.05B

Pharma 2026

~6.5% CAGR through 2030 · ~98% imported

\$680–760M

Devices 2026

GCC devices report band · MOPH regulated

The access pathway runs through committees, not insurers. MOPH issues marketing authorisation (WHO PQ or EMA / FDA / MHRA / TGA reference abridges the 12–24-month review). Then HMC's Pharmacy & Therapeutics Committee (covering 12 HMC sites including NCCCR), a separate Sidra Medicine P&T for paediatric and genomic indications, the MOPH National Drug Committee health-economic review for high-cost assets, and Health Holding Company annual INN

tenders — single winner per molecule. Private routes (Al Ahli, Aster) can run in parallel. Launch-to-access: **18–36 months**. Qatarpharm is the primary domestic producer.

Therapy 2026

Where the value sits

- **Oncology** USD 150–200M (12% CAGR) — NCCCR, Sidra paediatric oncology, genomics platform
- **Diabetes & metabolic** USD 120–160M (13.5% CAGR)
- **Cardiovascular** USD 100–140M (10%) — Heart Hospital volumes
- **Immunology & biologics** USD 90–130M (11%)
- **Respiratory** USD 60–90M (9%) — dust environment burden

Hospitals

Reference accounts

Hamad General (HMC)	750 beds
Trauma / main tertiary referral	
NCCCR (HMC)	200 beds
Oncology + stem cell transplant	
Sidra Medicine	400 beds
Paediatrics, genomics, women's health	
Heart Hospital (HMC)	174 beds
Cardiac surgery, EP, heart failure	
Al Rumailah (HMC)	550 beds
Rehabilitation / long-term care	

Epidemiology & Capacity

T2DM **24.6% age-standardised** (IDF 11th ed., 2024 — 5th highest globally); obesity **42% of adults — highest in the GCC** (Qatar STEPS 2022); cancer ~2,000 new cases/year (NCCCR 2023). ~3,200 beds (1.1 per 1,000); ~15,000 physicians (5.3 per 1,000 — the GCC's densest physician coverage). Qatar is piloting a formal HTA framework modelled on NICE and HAS — HTA-style dossiers are increasingly expected for oncology biologics and rare-disease therapies before HMC listing.

bionixus.com/qatar-healthcare-market-report

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COUNTRY 4 OF 6

Kuwait

Estimated at USD 7.5–8.5 billion in 2026 (USD 11–13 billion by 2030). Government is ~78% of spend at 5–6% of GDP. Kuwait runs one of the GCC's most centralised procurement systems: about 85% of pharmaceutical spend flows through Central Medical Stores, and single-winner tenders cascade access decisions across all government facilities simultaneously.

4.8M Population · ~69% non-nationals GDP per capita USD 32,000 (IMF 2025)	\$7.5–8.5B Healthcare 2026 → USD 11–13B by 2030 · government ~78%	\$0.95–1.1B Pharma 2026 Generics >70% by volume · ~95% imported	\$750–850M Devices 2026 GCC devices report band · CMS ~85% of volume
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The clock: DGPA (MOH) reviews the CTD over 12–24 months; referencing an existing SFDA or MOHAP registration under the GCC CTD framework materially shortens it. Pricing committee adds 2–4 months (cost-plus with price ceiling). CMS formulary listing, then annual single-winner tenders where price dominates. KOC and Kuwait Airways run smaller parallel employee formularies. Launch-to-access: **24–45 months**.

Vision 2035 targets ~30% private healthcare share (from ~15%), 11 new government hospitals, the Jaber Al-Ahmad expansion and Sabah Al-Ahmad Medical City. The Al-Afya scheme covers expatriates at designated facilities; private plans feed premium hospitals such as Royale Hayat faster than the CMS cycle.

EPIDEMIOLOGY & DEMAND The heaviest metabolic burden in the GCC - T2DM 25.6% age-standardised — 3rd highest globally (IDF 11th ed., 2024; crude prevalence 28.4%) - Obesity 47% of adults — among the highest worldwide (WHO 2022) - CVD ~38% of deaths (MOH Kuwait 2023) — the GCC's highest share - Fastest therapy growth: oncology (KCCC, Chest Disease Hospital), metabolic intensification, rare disease (consanguinity), immunology. CV remains largest by volume. ~7,500 beds (1.6 per 1,000) · ~20,000 physicians (4.1 per 1,000)	HOSPITALS Reference accounts <table><tr><td>Mubarak Al-Kabeer</td><td>700 beds</td></tr><tr><td colspan="2">Cardiology, nephrology, tertiary</td></tr><tr><td>Al Sabah</td><td>550 beds</td></tr><tr><td colspan="2">Trauma / emergency</td></tr><tr><td>Kuwait Cancer Control Centre</td><td>—</td></tr><tr><td colspan="2">Oncology / transplant reference</td></tr><tr><td>Adan</td><td>450 beds</td></tr><tr><td colspan="2">South Kuwait tertiary</td></tr><tr><td>Al Amiri</td><td>350 beds</td></tr><tr><td colspan="2">Internal medicine / GI</td></tr><tr><td>Royale Hayat</td><td>130 beds</td></tr><tr><td colspan="2">Premium private</td></tr></table>	Mubarak Al-Kabeer	700 beds	Cardiology, nephrology, tertiary		Al Sabah	550 beds	Trauma / emergency		Kuwait Cancer Control Centre	—	Oncology / transplant reference		Adan	450 beds	South Kuwait tertiary		Al Amiri	350 beds	Internal medicine / GI		Royale Hayat	130 beds	Premium private	
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GCC RECOGNITION — THE LIMIT

GCC mutual recognition helps the DGPA review. It does **not** replace MOH pricing or CMS tendering. Winning the molecule lot is still the public-channel P&L.

Oman

Estimated at USD 5.5–6.5 billion in 2026 (~6.2% CAGR through 2030). Government is ~70% of spend at ~5.5% of GDP. MOH Central Pharmacy supplies government hospitals; private pharmacies and groups (Aster, Badr Al Samaa) take the rest. Vision 2040's Dhamani insurance expansion is the retail-channel thesis — not a UAE-style private glide path overnight.

~5.0M**Population 2026**

NCSI Oman · GDP per capita ~USD 20,000 (IMF 2025)

\$5.5–6.5B**Healthcare 2026**

~5.5% of GDP · government ~70% of spend

\$520–580M**Pharma 2026**

6.5–7% CAGR · ~97% imported

\$280–320M**Devices 2026**

GCC devices report band · MOH Oman regulated

MOH Drug Registration & Control: CTD dossier, GCC mutual recognition applicable, technical review often 18–30 months, then pricing (2–4 months), marketing authorisation, Central Pharmacy / CSSD listing (3–6 months) and annual tenders. Dual MOCIP import licensing plus clinical facility credentialing lengthens monoclonal cold-chain onboarding when Muscat logistics are disrupted. Launch-to-access: **24–45 months**. Sultan Qaboos University Hospital is the academic gatekeeper for early neurology DMT switches.

HOSPITALS**Reference accounts****Royal Hospital Muscat** **600 beds**

National referral; oncology, cardiology, neurology

SQUH **500 beds**

Academic; genomics, neurology research

Khoula Hospital **500 beds**

Level 1 trauma, orthopaedics

National Oncology Centre **—**

Radiotherapy / chemotherapy — Royal Hospital campus

Al Shifa · Muscat Private **170 / 120 beds**

Private; Dhamani-linked reimbursement

CAPACITY & EPIDEMIOLOGY**System profile**

- ~7,000 beds (1.4 per 1,000) · ~14,000 physicians (2.8 per 1,000)
- T2DM 17.0% age-standardised (IDF 11th ed., 2024) — lowest in the GCC, but 50% of cases undiagnosed
- CVD 28% of deaths (MOH Oman 2023)
- Cancer ~3,500 new cases/year — colorectal and breast most prevalent (Oman National Cancer Registry 2022)

GROUND TRUTH

Interior mountain transport still delays STEMI cath-lab arrival versus coastal Muscat — a device and thrombolytic logistics issue, not a "small market" abstraction.

bionixus.com/oman-healthcare-market-report

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COUNTRY 6 OF 6

Bahrain

Estimated at USD 2.5–3.0 billion in 2026 (~5.5% CAGR through 2030). Government ~65% of spend at ~6.5% of GDP; the private share is rising with insurance and medical tourism. Bahrain's primary value in a GCC strategy is regulatory efficiency: NHRA's mutual-recognition pathway accepts SFDA and MOHAP registrations, cutting time and cost — while pricing and MOH tenders remain fully Bahraini.

1.63M Population 2026 CIO Bahrain · GDP per capita ~USD 22,000 (IMF 2025)	\$2.5–3.0B Healthcare 2026 ~6.5% of GDP · ~1,800 beds (1.1 per 1,000)	\$420–480M Pharma 2026 Almost entirely imported	\$220–280M Devices 2026 GCC devices report band · NHRA regulated
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Pathway: NHRA application (eCTD / GCC CTD) → technical review typically 12–18 months standalone, or **3–9 months via SFDA / MOHAP mutual recognition** → pricing unit 2–3 months → marketing authorisation → MOH Central Pharmacy listing covering Salmaniya Medical Complex and BDF Hospital (3–6 months) → national tender. Launch-to-access: **18–30 months**. Salmaniya–BDF formulary synchronicity is unusually tight versus other GCC pairs. Bahrain was the first GCC state with a formal biosimilar pathway, and NHRA listings are watched by neighbouring authorities — an early positive listing can support pricing discussions elsewhere in the Gulf.

HOSPITALS

Reference accounts

Salmaniya Medical Complex **1,200 beds**
Main tertiary; oncology, cardiology, nephrology

King Hamad University Hospital **380 beds**
JCI academic tertiary

BDF Hospital **350 beds**
Military–civilian tertiary

American Mission Hospital **140 beds**
Oldest private hospital in the Gulf

Awali Hospital **150 beds**
Private

CAPACITY & EPIDEMIOLOGY

System profile

- T2DM 22.1% age-standardised — 10th highest globally (IDF 11th ed., 2024)
- CVD 32% of deaths (MOH Bahrain 2023)
- Cancer ~900 new cases/year — breast and colorectal lead (National Cancer Registry Bahrain)
- Elevated genetic disease burden (thalassaemia, familial hypercholesterolaemia) supports rare-disease and lipid programmes at KHUH

MODELLING NOTE

Eastern Province (Saudi) commuter leakage distorts inpatient-day attribution if residency is assumed from claims geofences. Model both directions.

Six-country comparison

One table for the board pack: size, payer mix, tender anchor and the realistic access clock, side by side. Device bands follow the August 2026 GCC devices report; healthcare and pharma bands follow each published country report.

	KSA	UAE	QATAR	KUWAIT	OMAN	BAHRAIN
Population	37.2M	10.3M	2.84M	4.8M	~5.0M	1.63M
Healthcare 2026	\$55–65B	\$30–33B	\$8.5–10B	\$7.5–8.5B	\$5.5–6.5B	\$2.5–3.0B
Pharma 2026	\$8.5–9.5B	\$3.8–4.5B	\$0.90–1.05B	\$0.95–1.1B	\$520–580M	\$420–480M
Devices 2026	\$4.4–4.8B	\$2.2–2.5B	\$680–760M	\$750–850M	\$280–320M	\$220–280M
T2DM (IDF 2024, age-std)	23.1%	20.7%	24.6%	25.6%	17.0%	22.1%
Public vs private	Gov ~62%	Private ~65%	Gov ~80%	Gov ~78%	Gov ~70%	Gov ~65%
Tender anchor	NUPCO	Emirate + insurers	HMC / HHC	CMS	MOH Central	MOH tenders
Access clock	36–54 mo	18–30 mo	18–36 mo	24–45 mo	24–45 mo	18–30 mo

All six device bands are the GCC medical devices report shares of the \$9.0B regional total (August 2026 update) and roll up to it. Pharma bands are country-report prescription-market scopes; do not add them and expect the ~\$27B full-channel GCC rollup — scope and vintage differ.

HOW BIONIXUS TYPICALLY SEQUENCES A GCC LAUNCH STUDY

1 · **SFDA + NUPCO analogue** (largest P&L). 2 · **MOHAP with parallel DHA/DOH** (fastest private mix). 3 · **HMC / NCCCR** if oncology or paediatric genomics. 4 · **CMS** if public-volume generics or hospital injectables. 5 · **Oman / Bahrain as follower markets** using GCC reference — unless a named account (Royal Hospital, SMC) is the buyer.

Methodology & sources

Figures in this sample are taken from BioNixus published website intelligence — country healthcare market reports, the GCC Pharma Market Report 2026, the GCC medical devices report and GCC market-intelligence KPIs — as of August 2026, and validated against the cited public sources below. Where a range is shown, it is the published BioNixus band, not a single-point vendor forecast.

PRIMARY CONSTRUCTION — FULL PROGRAMMES, NOT THIS PDF

How the underlying data is built

Hospital procurement monitoring (NUPCO, CMS, HMC/HHC, MOH), pharmacy dispensing records, bilingual physician-reported prescribing panels, formulary-committee interviews and tender-notice analogues — reported at country,

CITED SECONDARY SOURCES

External evidence base

- **GCC devices rollup** — USD 9.0B (2026) → 16.2B (2035), ~6.7% CAGR, as cited in the BioNixus GCC medical devices report (August 2026); consistent with the Towards Healthcare 2026–2035 databook (USD 9.01B → 16.19B at 6.73% CAGR)

hospital, department, indication and patient level. This sample does not reproduce those microdata tables.

- **IDF Diabetes Atlas, 11th edition** (2024 estimates) — adult T2DM prevalence and undiagnosed shares
- **IMF 2025** — GDP per capita · **World Bank / national statistics agencies** — population
- **MOH statistics and national cancer registries 2022–2024** — mortality and incidence
- **UAE NCD Survey 2022 · Qatar STEPS 2022 · WHO Kuwait Country Profile 2022** — obesity

RECONCILIATION RULES USED IN THIS EDITION

How conflicting published bands were resolved

- GCC devices total and all six country device shares follow /gcc-medical-devices-market-report (August 2026 update). Country healthcare pages still carrying older device KPI bands were superseded.
- Country healthcare and pharma bands follow each country healthcare market report FAQ, except UAE pharma, which uses the BioNixus market-intelligence KPI (USD 3.8–4.5B) for consistency.
- The GCC pharma rollup (USD 23.7B 2024 → ~27B 2026, 7.6% CAGR) follows the dedicated GCC Pharma Market Report 2026. It uses a broader full-channel scope and will not equal an arithmetic sum of country prescription-market midpoints.
- Diabetes prevalence uses the IDF Diabetes Atlas 11th edition (2024 age-standardised estimates), superseding earlier atlas figures still quoted on some country pages.

SCOPE CAVEATS

Read before quoting

- Competing vendor device totals diverge when digital therapeutics, dental-only channels or broader MENA geography are included — different scopes, not corrections of the same dataset.
- Market size ranges and CAGR bands are planning estimates unless tied to a named public dataset; validate regulatory and payer rules against live policy before implementation.
- Publication dates for core pages: country healthcare reports updated May 2026; GCC devices report modified August 2026; GCC market access guide February 2026. This sample edition: August 2026.

This document is a sample excerpt for gated download. Therapy SKU shares, tender calendars, and hospital-level consumption remain in commissioned BioNixus work. Nothing herein is legal, medical, pricing, or investment advice.

BioNixus

intelligence for Business Growth

Primary research, not a spreadsheet export. BioNixus is a healthcare and pharmaceutical market research firm founded in London in 2012 — US headquarters in Sheridan, Wyoming, with its founding London office, a Greater Cairo MENA hub, and field networks across all six GCC states. We run physician, payer and patient primary research plus market access and HEOR, calibrated to SFDA, MOHAP, NUPCO, HMC and CMS reality.

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PUBLISHED INTELLIGENCE

GCC Pharma Market Report 2026
GCC Medical Devices Market Report
Six country healthcare market reports
GCC Market Access Guide 2026

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